

Name: سومه محمد السيد شحابه
National ID: 27701011712321



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :
--:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Platinum plus gemcitabine chemotherapy

الوقت :

--:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



سوميه
محمد السيد شحاته
الكوم الأ خضر
مركز شبين الكوم - المنوفية

١٩٧٧/٠١ ٢٧٧.٠١.٠١ ١٧ ١٢٣ ٢١

LG9092838

٠١-٤٣٩٨١٦٥

٢٠٢٥/٠٧ ٢٧٧.٠١.٠١ ١٧ ١٢٣ ٢١



رية منزل
أنسى
ياسر عقل احمد عقل
البطاقة سارية حتى ٢٠٣٢/٠٧/٢٥



١٢



Code: 25960
Sex: Female
التاريخ: 9 - 1 - 2026م

اسم المريض: سومة محمد السيد
السن: 48 سنة
الطبيب المعالج: معهد الأورام

Clinical Data: Left axillary lymph node enlargement in a patient with history of managed right breast cancer.

Pathology Report

Nature of the specimen:

U/S guided fine needle aspiration of left axillary lymph node.

Gross Appearance:

Cytological smears and cell block were prepared.

Microscopic Appearance:

Cytological examination features moderately cellular smears showing multiple groups of atypical epithelioid cells admixed with many individual cells. The cells are large, moderate amount of amphophilic cytoplasm and large rounded nuclei with prominent nucleoli and irregular chromatin pattern. Background shows a large number of lymphocytes.

Diagnosis:

LEFT AXILLARY LYMPH NODE: U/S GUIDED FNA CYTOLOGY:

- **Cytological smears: Positive for malignant cells, consistent with metastatic carcinoma, of likely mammary origin.**

عبدالله محمد مصطفى
تحاليل الأنسجة والأورام

010 21 00 61 37 - 010 96 13 26 18

شبين الكوم - ميدان شرف - شارع اسكندر متفرع من شارع الأمين - برج الانوار المحمدية - الدور الثالث

جوسناف
رؤسني
٨
٩ - ٩٧مبادرة رئيس الجمهورية
لدعم صحة المرأة المصريةالهيئة العامة
للصحة
بمصر

09/23/2026

MDT FORM

Name:	سورة محمد السيد شحاته	MRN:	13713
Age:	50 Y	ID:	27701011712321
Menopausal Sta	post	Phone, #	01098233789
NRS SCORE:		Marital Status:	married with 3 offsprings
BMI:		Family History:	no
Gender		Reason Of MDT:	New Case
CO Morbidity:	Free	ECOG Performance Status	0 1 2 3 4
Surgical History:	hysterectomy 10 y ago , salpingoophorectomy		

Clinical Data:

BX:	
ER:	PR: HER2: KI67:

History:

55 Y/O F, known R BC dx 05/2024, initially LABC, ER-/PR-/HER2-, Ki67 ~20%. NAC: AC x4 ? T x4.
R BCS 03/2025 ? pCR. No adjuvant RT received.
Early ipsilateral recurrence 07/2025 (~4 mo later) ? R simple mastectomy + reconstruction ? IDC G2.
Subsequently developed L axillary LN enlargement ? Bx +ve for malignancy, received Gem/Carbo x4 cycles.
Repeat Bx revision : ER-/PR-/HER2- ? TNBC.
BRCA mutation +ve;
h/o of trauma (falling 2 times back 8/2026)
MRI LS spine 05/2026: degenerative changes/spondylosis, no definite osseous mets. MRI B/L hips 05/2026: NAD.
Bone scan 07/2026: ? tracer uptake L4-5, B/L pubic bones and ant. R. 5th rib.
PET/CT 09/2026: * Low-grade FDG uptake at R breast/operative bed, SUVmax ~2.5. * D10 sclerotic focus with low-grade FDG uptake, SUVmax ~2.5-2.9 ? indeterminate, close FU advised. * Diffuse uptake L5-S1, SUVmax ~5B, favored advanced degenerative/post-traumatic changes.
photo attached for breast

Sonomammography:

Biomarker/Molecular

Mets Workup:

Recommendations:

MDT Outcome & Plan

For MRI of the breast ?? inflammatory changes
Meanwhile, resume Gem/carbo with close monitoring of response.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. شيماء لاشين	د. شيماء لاشين		

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مركز النخبة للأشعة

Patient's Name : sanaa shaker gouda
Date : 21 July 2026
Prof Dr : Mohamed Elsenbawy

Left BREAST SONOMAMOGRAPHY

Known case of breast cancer underwent NACT:

- Mammogram shows upper outer quadrant clustered micocalcifications
- Ultrasound shows no masses or abnormal Densituies could be noted
- Dilated retroareolar ducts are seen largest about 3.3mm with anechoic content
- The skin, subcutaneous tissue, nipple and areolae are within normal
- Multiple enlarged (4) axillary lymph nodes with rounded contour and lost hila the largest measures 14x17mm

OPINION:

⇒ Case of breast cancer showed parital reopone with still noted micro:alcification and no masses could be noted on ULTRASOUND with reduction in size of the previously noted malignant lymph nodesBIRADs6

⇒ Rteroareolar simple ductecasia as described ...BIRAds2

RIGHT BREAST SONOMAMOGRAPHY

- Normal glandular & fibrous elements with no masses or suspicious calcifications could be seen.
- Normal skin thickness & subcutaneous tissue.
- Normal ductal system.
- Normal nipple.
- No pathologically enlarged axillary L.N. could be seen.

OPINION

Essential normal sono-mamography examination of the right breast.

Best Regards ;

Dr. Basma Elgendy.MD



الرنين المغناطيسي المغلق: شابين الكوم - ش عبد الفتاح فرج متفرع من ش الأمين - برج الأنوار المحمدية
٠١٠١٣٩٨٨٠٥٣ - ٠٤٨٢٣٣٣٣١٣ - ٠٤٨٢٣٣٣٣١٢ - ٠٤٨٢٣٣٣٣١١
الرنين المغناطيسي المغلق - PET- CT: شابين الكوم - ٣ ش الشهداء ميدان مجلس المدينة ٠٤٨٢٣٣٣٣١٩ - ٠١٠٠٤٣٧٧٤١٩
الأشعة المقطعية: شابين الكوم - ميدان شرف - ٨ ش الأمين ٠٤٨٢٣٣٣٣١٩ - ٠١٠٠٣١٢٨٥٥١
فرع مستشفى الجامعة برج حجازي شارع ياسين عبد الغفار بجوار مستشفى جامعة المنوفية ٠٤٨/٢٢٢٥١١٤ - ٠١٠٣٢٤٧٤٧١٩

معاً من أجل سلامتك



جامعة المنوفية
مستشفى معهد الأورام
وحدة الطب النووي



Tc ^{99m} Bone scintigraphy

الاسم	سومة محمد السيد شحاتة	٤٣ سنة
التاريخ	٢٠٢٦/٢/٧	F
	النوع	

TECNIQUE:

Whole body anterior & posterior scans taken 3 hours after IV injection of 20 mci of the tracer Tc ^{99m} (MDP.)

FINDINGS

- The current study revealed absence of any focal active osseous lesion that would account for bone metastatic.

OPENION:

- No bony metastasis could be detected by the scan .

Verified by:

Prof/Amira Ebrahim Aldesoky

Assistant Professor of clinical Oncology &
nuclear medicine

Clinical history:

- 48 years old patient with breast cancer, underwent reconstruction in received CTH that ended
- PET/CT requested for



managed right right CBS and 09/2025, 8 months ago. assessment.

Technique:

- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed using Siemens Biograph mCT simultaneously and used for attenuation correction, anatomical localization and diagnosis. Max. variant of SUV; a Semi-quantitative analysis would be done for selected ROI.s and the normal threshold is < 2.5 .

No previous PET/CT available for comparison,

Reference hepatic activity: 3.7

Loco-regional:

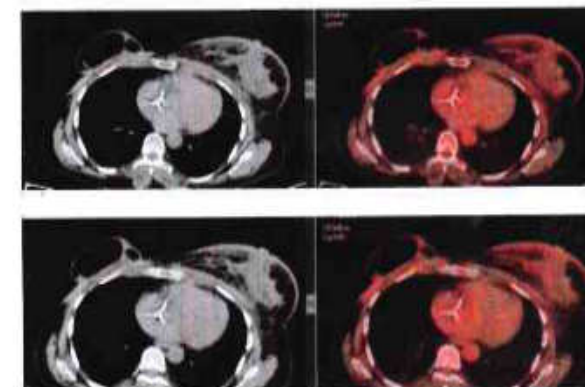
Right breast: Evidence of right CBS with reconstruction showing distorted parenchyma with low grade FDG activity reaching $SUV_{max} \sim 2.5$, yet No sizable FDG avid lesions detected,

Right axilla: No hypermetabolic right axillary LNs detected, non FDG avid CT detected subpectoral sub-cm LN reaching 6mm, for follow up



Left breast: Oedematous left breast parenchyma with mild overlying skin thickening reaching 1cm in max thickness with $SUV_{max} \sim 2.4$, for clinical correlation.

Left axilla: No hypermetabolic left axillary LNs detected.



Head and neck:

* Normal diffuse brain activity with no hypermetabolic brain lesions.

Report:

Name: Somaya Mohammad El-Sayed Date: 15/09/2026

Name: Somaya Mohammad El-Sayed Date: 15/09/2026

- * Physiologic FDG uptake is seen in the nasopharynx, oropharynx, salivary glands, and larynx, showing normal appearance.
- * Physiologic uptake is seen in submandibular and parotid salivary glands, showing normal appearance.
- * Normal size, enhancement, & symmetrical appearance of thyroid gland with No FDG avid lesions detected.
- * No hypermetabolic cervical lymphadenopathy detected.
- * No FDG avid neck soft tissue mass lesions.

Chest:

- * No hypermetabolic mediastinal lymphadenopathy.
- * No FDG avid lung nodules or masses detected.
- * No metabolically active pericardial or pleural lesions detected.
- * Physiologic FDG uptake is noted in the heart.
- * **Portacath noted.**

Abdomen and pelvis:

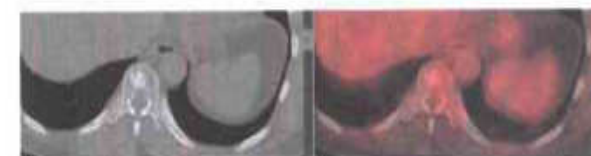
- * Physiologic diffuse FDG uptake is seen over average size liver with homogeneous parenchymatous texture. No FDG avid focal lesions detected. Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries. No intrahepatic or extra hepatic biliary radicles dilatation.
- * Physiologic diffuse FDG uptake is seen over average size spleen with homogeneous parenchymatous texture and no FDG avid focal lesions detected.
- * Normal size, parenchymal thickness, & density of both kidneys with no stones or back pressure changes.

Physiologic FDG excretion is seen in the kidneys and bladder.

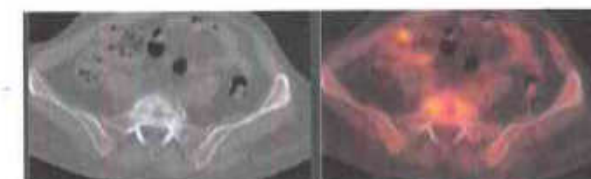
- * Normal appearance of the pancreas, aorta, IVC, and both supra renals with no FDG avid lesions detected.
- * No hypermetabolic abdominal or pelvic lymphadenopathy.
- * No FDG avid ascites or abdominal collections noted.

Musculoskeletal:

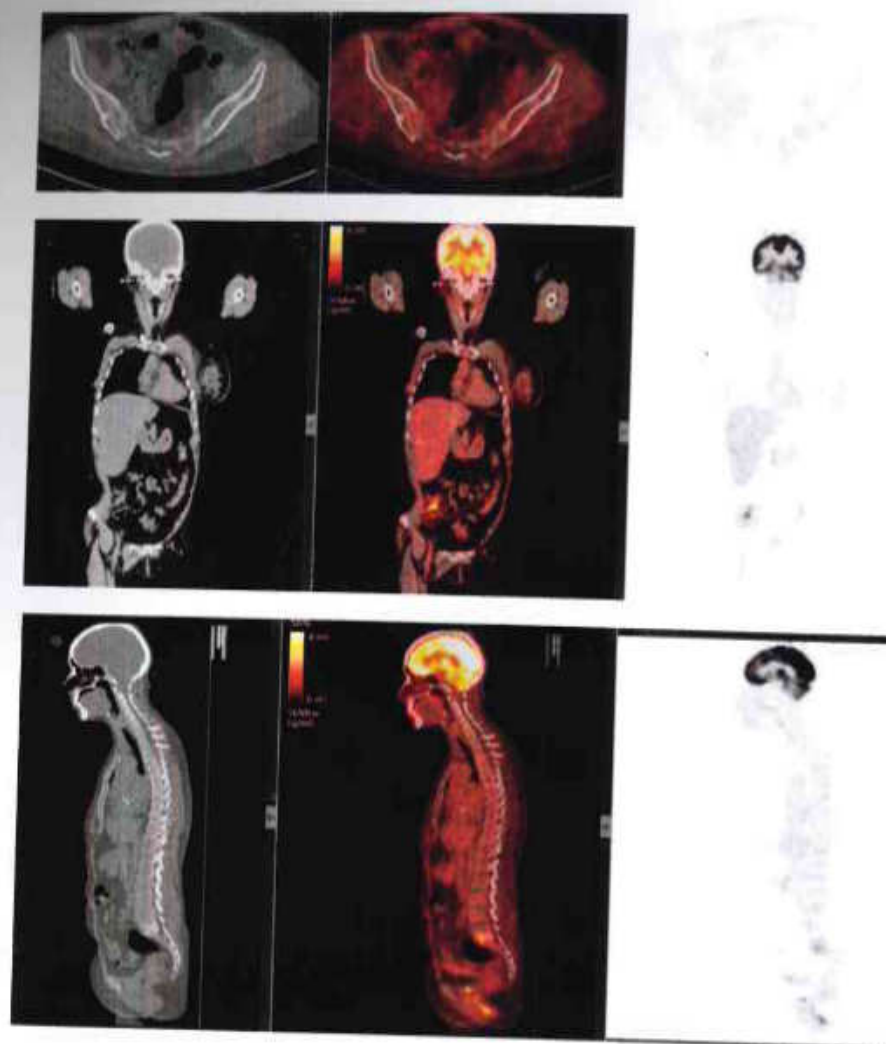
- * **Low grade FDG activity is seen over DV10 sclerotic focus with SUVmax~2.9, for close follow up**



- * **Diffuse FDG activity is seen over the LV5/S1 with SUVmax~5.3, likely of advanced degenerative nature/post traumatic for clinical correlation**



- * **Non FDG avid few right iliac bone sclerotic foci, likely benign for follow up**
- * No focal active lytic bone lesions detected.



Conclusion:

PET/CT study for a known case of managed right breast cancer revealed:

Considered Negative study for FDG avid residual right breast neoplastic disease or definite active metastatic deposits, however close follow up on lower dorsal vertebral sclerotic changes is still recommended to monitor their behavior over time.

*Thank you for referral,
Mayadah Ashraf, MSc
Dr. Walid Omar, MD
Nuclear Medicine
Prof. Of Nuclear Medicine*

Pat. Name: Soma Mohamed Al Sayed Shehata	Date : 31/05/2026
Ref. Dr : المركز الطبي الخيري بالبتانون	Code : 841469
Procedure Name: MRI Lumber Spine CLOSED	

PRE AND POST-CONTRAST MRI EXAMINATION OF THE LUMBAR SPINE

Technique of the Examination:-

- Sagittal T1 , T2 WIs- STIR
- Post -contrast series

Axial T2 & T1 WIs

Findings:-

- The alignment of the lumbar spine is preserved except for muscle spasm .
- Spondylotic changes of the lumbar spine:
Discs desiccation and decreased disc height .
Marginal osteophytosis.
- L4/5 mild diffuse posterior disc bulge is seen effacing the epidural fat , indenting ventral aspect of thecal sac and encroaching upon both exit foraminae with acute annular tear at posterior aspect of the disc .
- L5/S1 posterior central small focal disc protrusion on top of diffuse posterior disc bulge is seen effacing the epidural fat , indenting ventral aspect of thecal sac and encroaching upon both exit foramina .
- No areas of abnormal enhancement
- The apophyseal joints are of normal alignment with intact articular surfaces, No ankylosis or cysts formation.
- Normal thickness of the ligamentum flavum. No signs of canal stenosis.
- No intra or extra-dural soft tissue masses or collection detected.
- Normal MRI appearance of the conus medullaris region and cauda equina nerve roots.
- The vertebral bodies show normal height contour and preserved marrow signal with no destructive lesion.
- No abnormal para-spinal soft tissue intensities.
- Intact paraspinal musculature.

Opinion:-

- Mild Lumbar spondylosis
- L4/5 mild diffuse posterior disc bulge
- L5/S1 posterior central small focal disc protrusion on top of diffuse posterior disc bulge

Dr. Mohamed Shawky , MD

مودة كان

Pat. Name: Soma Mohamed Al Sayed Shehata	Date : 31/05/2026
Ref. Dr : المركز الطبي الخيري بالبتانون	Code : 841469
Procedure Name: MRI Pelvis CLOSED	

PRE AND POST-CONTRAST MRI BOTH HIPS, USING DIFFERENT PULSE SEQUENCES AND PARAMETERS, ON A SUPERCONDUCTIVE 1.5 TESLA MAGNET REVEALED :

TECHNIQUE:

-Axial T1WI & PDFS. - Coronal T1WI , T2WI &PDFS - Sagittal T2WI

-POST -CONTRAST SERIES

MRI FINDINGS:

- Normal size, shape and signal of both femoral heads with no evidences of avascular necrosis seen.
- Intact acetabula on both sides, with normal appearance of the acetabular labra and no evidence of labral tear.
- No remarkable joint effusion detected bilaterally with no intra-articular loose bodies.
- No areas of abnormal enhancement
- Normal MRI appearance of Hamstring origin, rectus femoris origin and adductors origin
- Normal MRI features of the periarticular soft tissues.
- Normal signal intensity of the visualized pelvic muscles with no evidences of tear or muscle strain.
- No gross intrapelvic soft tissue structures abnormalities observed.

Opinion:

Normal MRI both hip joints.

Dr: Mohamed Shawky MD

مودة كان

Pat. Name: Soma Mohamed Al Sayed Shehata	Date : 31/05/2026
Ref. Dr : المركز الطبي الخيري بالقاهرة	Code : 841469
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PRE AND POST-CONTRAST MRI BOTH HIPS, USING DIFFERENT PULSE SEQUENCES AND PARAMETERS, ON A SUPERCONDUCTIVE 1.5 TESLA MAGNET REVEALED :

TECHNIQUE:

-Axial T1WI & PDFS.

- Coronal T1WI, T2WI & PDFS

- Sagittal

T2WI

-POST -CONTRAST SERIES

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- Normal signal intensity of the visualized pelvic muscles with no evidences of tear or muscle strain.
- No gross intrapelvic soft tissue structures abnormalities observed.

Opinion:

Normal MRI both hip joints.

Dr: Mohamed Shawky MD

محمد شوقي

Pat. Name: Soma Mohamed Al Sayed Shehata	Date : 31/05/2026
Ref. Dr : المركز الطبي الخوري بالختارون	Code : 841469
Procedure Name: MRI Lumbar Spine CLOSED	

PRE AND POST-CONTRAST MRI EXAMINATION OF THE LUMBAR SPINE

Technique of the Examination:-

- Sagittal T1 , T2 WIs- STIR
- Post -contrast series
- Axial T2 & T1 WIs

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Opinion:-

- Mild Lumbar spondylosis
- L4/5 mild diffuse posterior disc bulge
- L5/S1 posterior central small focal disc protrusion on top of diffuse posterior disc bulge

Dr. Mohamed Shawky, MD

0260540

Patient Name : الأستاذة / مريم محمد السيد شحاته
Patient No. : 27701011712321
Path No. : SR-139/26
Sex : Female
Age : 49 Y
Referring Doc :

Request No. : 13004710
Request Date : 26/01/2026
Verification Date : 16/02/2026
Financial Category : فئة المرضى - مرضى
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

- HER2: Negative (score 0)
- Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

- Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells .IHC 0+:as defined as incomplete membrane reaction in less than 10 % of tumor cells . IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr.Wajaa Ibrahim Ghoneim
Dr.Heba El-Shafeey

Verified by :
Dr.Heba Hasan

Printing Date : 25/02/2026 15:18:15

Page 2 of 2

Printing By : Mrs. Mayada Hegazy



عنوان - القاهرة - مصر الطبية - مبنى القويعة - الطابق 4 - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mir El Qadema - Cairo, Egypt

**JUSTAVE/
ROUSSY**
HISTOPATHOLOGY

Patient Name : عبد الله محمد حسن بن علي / عبد الله
Patient No. : 27701011712321
Path No. : SR-139/26
Sex : Female
Age : 48 Y
Referring Doc :

Regional No. : 13004710
Request Date : 26/01/2026
Verification Date : 16/02/2026
Financial Category : فئة المرضى - Out Patient
Department : Out Patient
Registered by : Mrs. Maryada Haggazy

Pathology Report

Clinical Data

Left axillary lymph node enlargement in a patient with history of managed right breast cancer.

Nature of specimen

Block and slide revision of cell block .

Gross examination

One referred paraffin block labelled by (259600B) & three referred slides were received .

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:

HER2/NEU (4B5), GATA3, PR (1E2), ER (SP1).

Microscopic examination

- * Histological type: Metastatic malignant cells
- * IPT (Positive control showed appropriate reactivity) :
- * GATA3:Weak positive in tumor cells .
- * ER: No nuclear reaction in tumor cells (Allred score 0/8)
- * PR: No nuclear reaction in tumor cells (Allred score 0/8)
- * HER2: No membrane reaction in tumor cells (score 0)

Diagnosis

Left axillary lymph node, Block revision

-Cell block slide with metastatic malignant cells likely of breast origin

IPT:

- * ER: Negative (score 0/8).
- * PR: Negative (score 0/8).

Verified by :

Dr.Hoba Hasan

Printing Date : 25/02/2026 15:18:15

Page 1 of 2

Printing By : Mrs. Maryada Haggazy



جوسناف
رؤسني
٨
٩-٢٧مبادرة رئيس الجمهورية
لدعم صحة المرأة المصريةالهيئة العامة
للصحة
بمصر

09/23/2026

MDT FORM

Name:	سورة محمد السيد شحاته	MRN:	13713
Age:	50 Y	ID:	27701011712321
Menopausal Sta	post	Phone, #	01098233789
NRS SCORE:		Marital Status:	married with 3 offsprings
BMI:		Family History:	no
Gender		Reason Of MDT:	
CO Morbidity:	Free	New Case	
Surgical History:	hysterectomy 10 y ago , salpingoophorectomy		
	ECOG Performance Status ○ 0● 1○ 2○ 3○ 4		

Clinical Data:

BX:	
ER:	PR: HER2: KI67:

History:

55 Y/O F, known R BC dx 05/2024, initially LABC, ER-/PR-/HER2-, Ki67 ~20%. NAC: AC x4 ? T x4.
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BRCA mutation +ve;
h/o of trauma (falling 2 times back 8/2026)
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photo attached for breast

Sonomammography:

Biomarker/Molecular

Mets Workup:

Recommendations:

MDT Outcome & Plan

For MRI of the breast ?? inflammatory changes
Meanwhile, resume Gem/carbo with close monitoring of response.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. شيماء لاشين	د. شيماء لاشين		

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